

WHAT THE GOVERNMENT KNOWS AND DOESN'T TELL YOU

A Plain Language Companion to the
Information Asymmetry Report

THE INFORMATION YOU NEED EXISTS. YOU JUST AREN'T ALLOWED TO SEE IT.

The Alberta Disability System Breakdown

April 2026

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PAGE 1 — WHAT THIS GUIDE IS ABOUT



TWO ERAS. SAME LOCK. THE MECHANISM HAS CHANGED. THE FUNCTION HAS NOT.

This guide is the short version of a longer report.

The long report is called the *Information Asymmetry Report*. It is 49 pages. It has footnotes, court cases, and government documents. This is the plain language version. It covers the same ground, in simpler words.

The long report asks one question: **why is so much of what disabled Albertans need to know kept from them — and what does that pattern have in common with the way Alberta treated disabled people sixty years ago?**

From 1928 to 1972, Alberta sterilized about 2,832 people the Eugenics Board judged unfit. That law is gone. The mechanism that made it possible — telling disabled people nothing about decisions being made about their lives — is not gone. It is still how the system works.

What does “information asymmetry” mean?

It means one side of a relationship knows more than the other side, and uses the gap to control what happens. In Alberta, the government knows the rules. You are the one who has to live by them. The gap between what the government knows and what it tells you is not an accident. It is the design.

■ HOW TO EXPLAIN THIS

Try saying this: “There is a 49-page report on how Alberta keeps information from disabled people who need it. This is the short version. It is free. It can be read aloud to anyone who needs it.”

PAGE 2 — THE BIG IDEA, IN ONE BREATH



THE GOVERNMENT TELLS YOU THINGS WHEN IT NEEDS YOU TO KNOW THEM. NOT BEFORE.

Here is the pattern this guide describes. The government knows things about disability. It knows what supports exist. It knows the rules. It knows the wait times. It knows what happens if you apply, and what happens if you do not.

You will not be told most of these things until the moment the government needs you to know them.

You will not be told about FSCD until your child is diagnosed. You will not be told about AISH eligibility criteria until you apply. You will not be told about ADAP until they are putting you on it. You will not be told the new rules until the new rules already apply to you.

By that point, you have lost the ability to plan. You have lost the ability to push back. You are responding to a situation the government has already shaped.

The information always existed. The timing of when you got it is the whole point.

Why this matters

Real consent — the kind a free society is supposed to be built on — requires that you know what you are consenting to before you consent to it. A system that tells you the rules at the moment it asks you to follow them is not asking for your consent. It is informing you of a decision.

■ HOW TO EXPLAIN THIS

Try saying this: "In Alberta's disability system, you find out the rules at the moment they are already being used on you. By then, you cannot say no."

PAGE 3 — STAGE ONE — BEFORE YOU ARE BORN



THE INFORMATION EXISTS IN THE ROOM. THE PATIENT CANNOT SEE IT.

Routine prenatal care in Alberta screens for a small list of conditions: Down syndrome, Trisomy 18, Trisomy 13, and structural anomalies. That is the entire window most parents see.

What is not part of structured prenatal education in this province: autism, ADHD, learning disabilities, intellectual disability not connected to the few conditions screened for, FSCD wait times, PDD eligibility thresholds, paternal genetic factors, or the basic vocabulary you would need to recognize developmental difference when it appears.

All of that information exists. The province has it. The medical system has it. Researchers have it. **It is not built into the public information you encounter when you become a parent.**

When your child later misses milestones, your family doctor or pediatrician will tell you, accurately, that “some children develop later.” By the time the developmental difference is named, the early intervention window has often closed and the years-long FSCD wait has not yet begun.

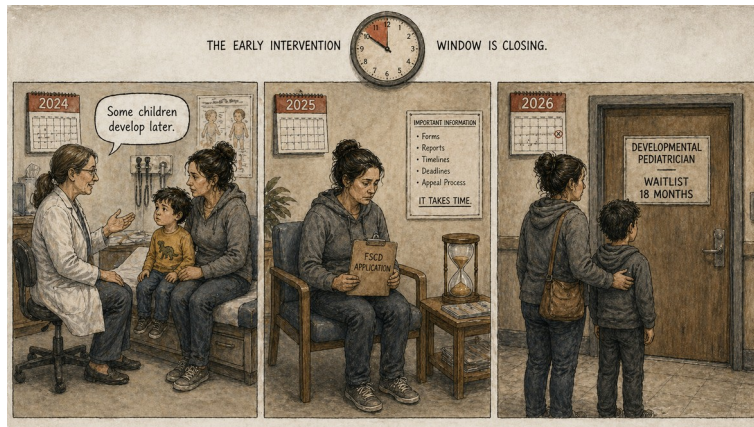
Why this is part of the pattern

The province does not have to lie about disability information for the asymmetry to operate. It only has to choose not to put the information into the channels parents actually encounter. The information stays in the room. The patient does not see it. The cost is borne by the family that walks into the gap.

■ HOW TO EXPLAIN THIS

Try saying this: “Alberta’s prenatal education only covers a few conditions. Most disabilities are not part of it. The information exists. Parents are not given it before they need it.”

PAGE 4 — STAGE TWO — WHEN YOUR CHILD IS YOUNG



THE WAIT IS THE WHOLE STORY. BY THE TIME THE DOOR OPENS, THE WINDOW HAS CLOSED.

Family doctors and general pediatricians are not trained to assess developmental disability at the level of detail families need. That is not a personal failing. It is a structural fact about Canadian medical training.

Specialist developmental pediatric assessment in Alberta is constrained by a number of practitioners radically below demand. Wait times stretch into months and, in many zones, years. The general pediatrician decides whether a referral is even made.

While the family waits, the early intervention window — the years between roughly 2 and 5 — is closing. The Inclusion Alberta survey of 540 families found the average wait for full FSCD services is **three years**. A child diagnosed at four whose family applies the same day will not, on average, see FSCD-funded supports until age seven. The program cannot, as it currently runs, deliver supports during the years its own evidence base says matter most.

Then comes age 18. FSCD ends. PDD does not automatically continue from FSCD — it is a separate program with separate criteria, and many disabled young adults do not qualify for PDD at all. Families discover this in the months around their child's eighteenth birthday. There is no continuity-of-service mechanism between the two programs.

What you would need, and do not get

A list of developmental milestones with the specificity to recognize concerns early. The criteria specialists actually use to decide whether to refer. The FSCD wait time, by zone, before you apply. The PDD eligibility threshold, before your child turns 18. None of these are published in any form a family can use in advance.

■ HOW TO EXPLAIN THIS

Try saying this: "Families learn what they needed to know after they needed to know it. The wait time is the system. By the time the help comes, the years it was meant for are gone."

PAGE 5 — STAGE THREE — SAME FILE, DIFFERENT DOOR



TWO PEOPLE WITH THE SAME FILE GO TO DIFFERENT DOORS. THE FILE IS NOT WHAT IS BEING ASSESSED.

On July 1, 2026, the Alberta Disability Assistance Program (ADAP) replaces AISH for about 50,000 of the roughly 79,290 people on AISH today. The rest are exempt. Here is what the exemption framework reveals.

Four groups stay on AISH automatically: people in the PDD program (about 13,641), people over 60 (about 13,603), residents of continuing care (about 2,423), and people with palliative or terminal conditions (count not published).

For the exempt groups, the existing AISH file — the same medical reports, by the same doctors, signed off by the same provincial assessors — is treated as conclusive. **No reassessment. No new medical report. No Medical Review Panel. The file is enough.**

For the transitioning group, the same kind of file, produced by the same system, is treated as not enough. They are moved to ADAP by default. The only way back to AISH is a new Disability Assistance Medical Report and a new appearance before a Medical Review Panel hired by the same ministry whose budget benefits from each ADAP placement. Bill 12 has eliminated the independent appeal pathway for this group.

Two people with substantively identical files receive structurally different treatment. The variable is not the file. The variable is the category the government has assigned to the recipient.

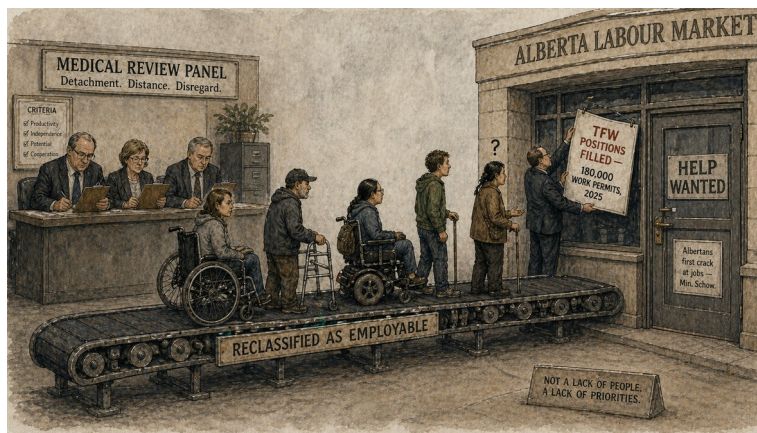
What the contradiction means

Each exempt group is exempt because employment is not plausible for them — too old, too developmentally disabled, in continuing care, or dying. The Medical Review Panel is therefore not assessing whether your disability is real. The exemptions already concede that. The Panel is assessing whether you can be plausibly framed as someone who could be moved into employment. The criterion has not been published.

■ HOW TO EXPLAIN THIS

Try saying this: “This is not a disability assessment. It is a labour-market sort. Same file, different door — and the door you go through depends on whether the government thinks it can call you employable.”

PAGE 6 — STAGE THREE CONTINUED — A LABOUR MARKET YOU CANNOT GET INTO



RECLASSIFIED AS EMPLOYABLE — INTO A LABOUR MARKET THE SAME GOVERNMENT HAS LEGISLATED AS INACCESSIBLE.

The Alberta unemployment rate in February 2026 was 6.3%. Full-time employment dropped by 17,300 positions in that single month. Youth unemployment is 14.5%. The employment rate for people with severe disabilities — the group most comparable to AISH recipients — is 26.4% nationally. This is the labour market the Medical Review Panel's reclassified population is being directed into.

On April 1, 2026, the same government introduced the Immigration Oversight Act. Minister of Jobs Joseph Schow said the law was about ensuring that "Albertans have first crack at Alberta jobs."

That statement is the government's own admission that Albertan workers are being structurally bypassed in their own labour market. Work permit holders in Alberta went from about 45,000 in 2021 to about 180,000 in 2025 — a 300% increase. In business, finance, and administration positions — the office work most accessible to people who can sit at a desk — Temporary Foreign Worker positive Labour Market Impact Assessments grew from 166 in 2018 to 2,242 in 2023. That is a 1,251% increase. The government's stated reason: employers "cannot find the employees in Alberta."

These two positions cannot both be true at the same time. A government that legislates because Albertans are being bypassed for their own jobs cannot simultaneously claim that 50,000 disabled Albertans, with documented permanent severe disabilities, will encounter a labour market capable of absorbing them.

The reclassification is administrative. The employment is hypothetical. The income reduction is real.

This is not anti-immigration

The structural problem is that Alberta employers have built around accommodation costs, productivity expectations, and hiring practices that exclude disabled workers. ADAP does not address those barriers. ADAP just calls the disabled population employable and sends it into a market the government has already named as failing to absorb its non-disabled job-seekers.

■ HOW TO EXPLAIN THIS

Try saying this: "The same government that says Albertans cannot get jobs in Alberta is reclassifying disabled people as employable. Both things cannot be true. One of them is not."

PAGE 7 — STAGE FOUR — WHAT MADE YOU SICK



THE WARNINGS EXIST. THEY FACE THE WRONG WAY.

A substantial number of disabled Albertans became disabled because of exposure to substances and conditions whose harmful effects were known to industry, government, and medical institutions before, during, or shortly after the exposure happened.

The science on asbestos was clear by the 1960s. The science on lead was clear by the 1970s. The science on PFAS was clear by the 1980s and 1990s. The information existed. It moved from research literature, to clinical practice, to public health education, to individual patient awareness — at each stage, late. By the time an exposed person can name the framework that explains their cluster of conditions, they have been failed by the public health information system for the duration of an adult lifetime.

The same pattern operates across veterans whose service exposures took decades to be acknowledged, oil and gas workers exposed to volatile organics and silica, the population exposed to leaded gasoline emissions during decades of leaded fuel use, and the First Nations communities still being failed on contaminated water. None of these populations were given the information that would have let them recognize, prevent, or mitigate the harm.

The disability income support process does not invite you to document any of this. The AISH application asks you to document your disability in clinical terms. It does not ask you to document the institutional failures that produced it. The province takes no part of the responsibility into the adjudication. You approach the program as a suppliant, not as a citizen whose disability the institutional architecture of the province participated in producing.

Why this stage matters

Disability is not always a condition that simply exists. For many people, it is a condition produced — in part or whole — by what institutions did or failed to do. The information that would have prevented the harm was held by the institutions that caused it. The supports offered afterwards are calibrated to poverty. Bill 12 reduces them further.

■ HOW TO EXPLAIN THIS

Try saying this: “The province had information that could have prevented the disability. It did not share it. It now offers the smallest possible support — and is currently reducing it. That is the architecture of the harm.”

PAGE 8 — STAGE FIVE — WHAT HAPPENS AFTER



THE INFORMATION THAT WOULD LET PARENTS PLAN DOES NOT EXIST. THE FOG IS INSTITUTIONAL, NOT PERSONAL.

This is the stage that wakes parents at three in the morning. What happens to a disabled adult after the parent or partner who has been holding their life together is gone?

What happens depends on a set of variables a caregiver, in most cases, cannot fully evaluate during their lifetime. Whether the disabled adult is in PDD. Whether residential supports exist in their region. Whether the placement maintains community, family, and routine — or disrupts all three for administrative convenience. Whether the family has the resources, time, and institutional knowledge to advocate during the transition. By definition, the family no longer has any of those things, because the person who held them has died.

The province does not publish a plain-language guide for caregivers on this. The Office of the Public Guardian and Trustee does not publish operational standards. The PDD program does not publish typical placement timelines or the criteria by which one residential setting is chosen over another. Disability succession planning is not in the standard scope of legal aid in Alberta. The cost of a private lawyer with this expertise is not subsidized.

The result is a population of disabled Albertans whose post-caregiver futures are differentiated along class lines. A family with money for an estate planner gets one future. A family without those resources gets a different one. The province's obligation to the disabled adult — as a citizen — is identical in both cases. The institutional environment they will encounter is dramatically different. The information vacuum is the mechanism that produces the difference.

There is also an institutional reason the silence persists. The province's residential and supported-living capacity cannot, at standards families would consider acceptable, absorb the cohort of disabled adults whose caregivers will die in the coming decades. Families who do not know what their disabled adult's post-caregiver future will look like cannot organize politically around the inadequacy of that future. Families who learn the answer at the moment of their own incapacitation can no longer organize at all. **The information vacuum is operating, at this stage, as a mechanism of political demobilization.**

■ HOW TO EXPLAIN THIS

Try saying this: "The province has not told caregivers what will happen to their disabled family member after they die. That silence is doing work. Families who do not know cannot organize. Families who learn at the end cannot organize anymore."

PAGE 9 — A NOTE ABOUT MAID

STRUCTURAL COERCION IS NOT THE SAME AS INTENT. THE ABSENCE OF AN IMPACT ASSESSMENT IS THE STORY.

This page is shorter than the others, and there is no illustration. The subject does not lend itself to one. Since the March 2021 federal expansion of medical assistance in dying (MAID) to people whose natural death is not reasonably foreseeable, the deliberate ending of one's life has been an institutionally available option for disabled Canadians whose conditions are not terminal.

Health Canada's annual monitoring reports document a population of so-called Track 2 MAID requests in which financial precarity, housing instability, and inadequate support are cited by the recipient as the primary drivers of the request — not the underlying medical condition.

Major Canadian disability organizations — including Inclusion Canada and the Council of Canadians with Disabilities — and the United Nations Special Rapporteur on the Rights of Persons with Disabilities have characterized the intersection of expanded MAID eligibility with reduced disability supports as a circumstance of **structural coercion**.

A government that proceeds with reductions to disability supports in the face of this documented pattern, without conducting a public health impact assessment of the foreseeable interaction between the policy change and disability-related MAID requests, has accepted the foreseeable outcome as a feature rather than a defect of the policy.

What this paragraph is, and is not

It is not a claim that the Government of Alberta intends people to die. It is a claim that the government has not assessed the foreseeable interaction between its policy and a documented pattern, and that the absence of that assessment is itself the story. The structural-coercion framing used here is the framing developed by the disability organizations named above. This guide is adopting it, not originating it.

■ HOW TO EXPLAIN THIS

Try saying this: "This is not about intent. It is about what the province has not assessed. Major disability organizations and the UN have called this pattern structural coercion. The province has not done the assessment that would respond to that."

PAGE 10 — WHAT YOU ARE OWED



THESE ARE NOT POLICY PREFERENCES. THESE ARE OBLIGATIONS THE PROVINCE HAS ALREADY ACCEPTED.

The Canadian Charter of Rights and Freedoms. Section 7 forbids deprivation of life, liberty, or security of the person on the basis of evidence the person cannot see, criteria they cannot review, or processes they cannot meaningfully participate in. Bill 12 does all three to about 50,000 people. Section 15 guarantees equal benefit of the law; *Eldridge* (1997) said that means substantive equality, not just identical text on paper.

The UN Convention on the Rights of Persons with Disabilities. Canada ratified the CRPD in 2010. Article 4(3) requires close consultation through representative organizations. No major Alberta disability organization recognizes the Bill 12 consultation as meeting that standard. Articles 12, 13, 19, 21, and 28 — equal recognition before the law, access to justice, community living, accessible information, an adequate standard of living — are each implicated.

The UN Committee on the Rights of Persons with Disabilities issued its Concluding Observations on Canada on March 26, 2025 — nine months before Bill 12 passed. It recommended that Canada condition fiscal transfers to provinces on adherence to the CRPD. Canada has not acted. Bill 12 is the direct consequence.

The Alberta Bill of Rights. The province's own foundational rights instrument, predating the Charter by more than twenty years, protects at section 1(a) the right not to be deprived of liberty, security, or property except by due process of law. The AISH-to-ADAP transition meets none of these. The province is violating its own rights instrument.

What the province owes, concretely

The full operational rules of ADAP in plain language, before transition. The eligibility criteria and assessment standards used by medical assessors, before assessment. The supplementary health benefits formulary, before consent. The restoration of independent appeal. Substantive consultation under CRPD Article 4(3) before any further reform.

■ HOW TO EXPLAIN THIS

Try saying this: “These are not asks. They are obligations — Charter, CRPD, Alberta Bill of Rights — the province has signed on to and chosen not to be held to. That is what we are naming.”

PAGE 11 — THIS REPORT IS THE NAMING



THE MECHANISM HAS SHIFTED. THE FUNCTION HAS NOT. NAMING IT IS THE FIRST STEP.

The Sexual Sterilization Act operated under an explicit framework: that disabled people were not capable of making decisions about their own lives, and that the institutions of the province were therefore entitled to make those decisions for them. The legal architecture of that framework was repealed in 1972. The operational principle it expressed — that disabled Albertans, and the families who support them, are not the primary users of the information governing their own lives — has continued to operate without modification across every disability program and every life stage examined in the long report.

The mechanism has shifted. The function has not.

The information asymmetry the province maintains is not a bureaucratic accident, an unfunded mandate, or a defensible administrative simplification. It is the operating principle of a system designed to govern a population whose participation it does not seek, whose consent it does not require, and whose fate after the death of those who advocate for them most fiercely it has no operational obligation to disclose.

This guide is the naming. The long report is the receipts.

Where to find the full report and the campaign

The full Information Asymmetry Report (49 pages, with footnotes and full sources) is available at the campaign website: albertadisabilitysystembreakdown.netlify.app. The campaign Facebook group is The Alberta Disability System Breakdown. Companion documents in the same series include the Charter of Rights Plain Language Guide, the Indigenous Disability Plain Language Guide, the Compound Failure series, and Government's Case Against Itself.

■ HOW TO EXPLAIN THIS

Try saying this: "This is the short version. The long version is online and is free. Share both. Print both. Read them aloud. Send them to your MLA, your MP, your federal disability minister. The naming is the first step."